Plans of Safe Care: State-by-State Summary

How each state and the District of Columbia carries out the federal Plan of Safe Care requirement for infants with prenatal substance exposure and their families. From planofsafecare.org, maintained by Margaret Lloyd Sieger, PhD, MS, University of Kansas School of Medicine. Generated November 13, 2026.

State policies change. If you know of an update for your state, please email msieger@kumc.edu.

Approach categories

- **Hospital-initiated.** Health care or community providers develop the plan during pregnancy or soon after birth, outside the child welfare system and before any decision about a report.
- **Dual Pathways.** Some exposures, such as medication for opioid use disorder, lead to a hospital-initiated plan. Other exposures require a report and a child protective services response.
- **All CPS Referrals.** Every screened-in report for prenatal substance exposure gets a plan, developed during the child protective services assessment whether or not a case is opened.
- **Alternative Response.** Plans are delivered through a voluntary, in-home child welfare track, without a formal investigation or court involvement.
- **Open Child Welfare Case.** Plans are developed only after a family is found to need child welfare intervention. The plan works as a safety plan or case plan.
- **No Policy.** No statewide Plan of Safe Care policy was identified as of 2024.

Categories reflect state policy as of 2024 (Lloyd Sieger et al., 2026), except Michigan, Oregon, Pennsylvania, Virginia, and Washington, which have been updated since. Bracketed numbers refer to the sources at the end.

Alabama

Profile last updated: October 2026

All CPS Referrals[1]. Every screened-in report for prenatal substance exposure gets a plan, developed during the child protective services assessment whether or not a case is opened.

How it works in practice[1]

Hospitals contact the Department of Human Resources whenever a doctor determines a newborn was affected by substance use or withdrawal. The plan is not a separate document. Caseworkers build it into the family's safety plan or case plan during the investigation or when a case is opened for services.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2018 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.

Purpose of required reports of drug use during pregnancy[3]	No reporting requirement identified
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	Yes
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of Alabama's Plan of Safe Care laws (Child Welfare Information Gateway)

Alaska

Profile last updated: October 2026

● **Open Child Welfare Case**[1]. Plans are developed only after a family is found to need child welfare intervention. The plan works as a safety plan or case plan.

How it works in practice[1]

The Office of Children's Services leads Plan of Safe Care work. Plans are developed for families who have an open child welfare case, as part of their case plan.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2016 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of Alaska's Plan of Safe Care laws (Child Welfare Information Gateway)

Arizona

Profile last updated: October 2026

● **All CPS Referrals**[1]. Every screened-in report for prenatal substance exposure gets a plan,

developed during the child protective services assessment whether or not a case is opened.

How it works in practice[1]
Arizona calls the plan an Infant Care Plan. The Department of Child Safety uses it for every substance-exposed newborn involved with the department. Caseworkers create it early, while deciding on the baby's safety, and update it at case planning and Child and Family Team meetings or when the baby's health needs change.

What the state calls the plan[1]	Infant Care Plan
Year the Plan of Safe Care policy began[1]	2018 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	Yes
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of Arizona's Plan of Safe Care laws (Child Welfare Information Gateway)

Arkansas

Profile last updated: October 2026

- **Open Child Welfare Case**[1]. Plans are developed only after a family is found to need child welfare intervention. The plan works as a safety plan or case plan.

How it works in practice[1]
The Division of Children and Family Services develops the plans. They are for infants reported to the child abuse hotline for prenatal substance exposure who are not found to be neglected.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2019 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Referral for assessment or treatment; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	Yes

Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of Arkansas's Plan of Safe Care laws (Child Welfare Information Gateway)

California

Profile last updated: October 2026

● **Open Child Welfare Case**[1]. Plans are developed only after a family is found to need child welfare intervention. The plan works as a safety plan or case plan.

How it works in practice[1]

Plans are developed for infants affected by substance use who are referred to child welfare, by either a medical professional or a child welfare worker. Hotline screeners record whether the infant was affected by fetal alcohol spectrum disorder, substance use, or withdrawal, and whether the family was referred to services.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2015 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral for assessment or treatment; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of California's Plan of Safe Care laws (Child Welfare Information Gateway)

Colorado

Profile last updated: October 2026

● **All CPS Referrals**[1]. Every screened-in report for prenatal substance exposure gets a plan, developed during the child protective services assessment whether or not a case is opened.

How it works in practice[8]

Colorado encourages starting the plan during pregnancy or at the birth hospital before discharge, with the parent and a team that can include health care, treatment, and peer support providers. Some hospitals send the state a notification without names, which is not a report to child welfare. A report is required only when there are safety concerns, and if a screened-in report has no plan, a caseworker creates one.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2021 (state law)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Public health data
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Colorado Department of Human Services: Plans of Safe Care FAQs · Summary of Colorado's Plan of Safe Care laws (Child Welfare Information Gateway)

Connecticut

Profile last updated: October 2026

● **Hospital-initiated**[1]. Health care or community providers develop the plan during pregnancy or soon after birth, outside the child welfare system and before any decision about a report.

How it works in practice[6]

Hospitals use an online portal to decide whether there is a safety concern. If there is none, they send an anonymous notification with no identifying information about the family. If there is a safety concern, they make a formal report to child welfare, which leads to an investigation. At the time of the study, Connecticut was reconsidering whether a positive infant drug test should be a reason to report.

What the state calls the plan[1]	Family Care Plan (formerly Plan of Safe Care)
Year the Plan of Safe Care policy began[1]	2019 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral for assessment or treatment; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	Yes

Learn more: Connecticut DMHAS: CAPTA Family Care Plans · Summary of Connecticut's Plan of Safe Care laws (Child Welfare Information Gateway)

Delaware

Profile last updated: October 2026

- **Alternative Response**[1]. Plans are delivered through a voluntary, in-home child welfare track, without a formal investigation or court involvement.

How it works in practice[1]

Hospitals notify the Division of Family Services about substance-exposed infants and infants with fetal alcohol spectrum disorder, and a plan is developed before the baby leaves the hospital. Delaware law says prenatal exposure alone is not abuse or neglect, but it leads to an investigation or family assessment. When a mother is taking a prescribed medication, hospitals can use a Medical Plan of Safe Care that the medical provider carries out and monitors.

What the state calls the plan[1]	Plan of Safe Care; Medical Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2018 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of Delaware's Plan of Safe Care laws (Child Welfare Information Gateway)

District of Columbia

Profile last updated: October 2026

- **Alternative Response**[1]. Plans are delivered through a voluntary, in-home child welfare track, without a formal investigation or court involvement.

How it works in practice[1]

We could not find a detailed public description of how the District of Columbia's process works. Under its approach, plans are developed through a child welfare track that offers voluntary, in-home services without a formal investigation.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2017 (state report)

Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	No reporting requirement identified
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	Yes
Child welfare accepts abuse or neglect reports during pregnancy[5]	Not identified
Child welfare accepts referrals for services during pregnancy[5]	Not identified

Learn more: Summary of District of Columbia's Plan of Safe Care laws (Child Welfare Information Gateway)

Florida

Profile last updated: October 2026

- **Alternative Response**[1]. Plans are delivered through a voluntary, in-home child welfare track, without a formal investigation or court involvement.

How it works in practice[1]

Plans are built into the family support or care plans made by whichever agency is working with the family. Florida's federal reports give few other details.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2018 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral for assessment or treatment; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	Yes
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	Yes

Learn more: Florida Department of Children and Families: Plan of Safe Care · Summary of Florida's Plan of Safe Care laws (Child Welfare Information Gateway)

Georgia

Profile last updated: October 2026

● **All CPS Referrals**[1]. Every screened-in report for prenatal substance exposure gets a plan, developed during the child protective services assessment whether or not a case is opened.

How it works in practice[1]

A plan is completed for every infant affected by prenatal substance exposure or fetal alcohol spectrum disorder, even when there is no specific allegation of maltreatment. The mother's health care provider can complete it during pregnancy or right after birth. When an allegation needs investigation, a DFCS case manager completes it.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2017 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral for assessment or treatment; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	Yes
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Georgia DFCS: Plans of Safe Care · Summary of Georgia's Plan of Safe Care laws (Child Welfare Information Gateway)

Hawaii

Profile last updated: October 2026

● **Open Child Welfare Case**[1]. Plans are developed only after a family is found to need child welfare intervention. The plan works as a safety plan or case plan.

How it works in practice[1]

Hawaii documents plans for substance-exposed infants in the Safe Family Home Report section of the family's case plan, the same tool it uses for all children in foster care.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2018 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	No reporting requirement identified

State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of Hawaii's Plan of Safe Care laws (Child Welfare Information Gateway)

Idaho

Profile last updated: November 2026

● **Open Child Welfare Case**[1]. Plans are developed only after a family is found to need child welfare intervention. The plan works as a safety plan or case plan.

How it works in practice[1][9]
Plans are developed within the child welfare system. Idaho's reports describe plans for a multidisciplinary committee to review services for these families and make recommendations to state and local providers. Idaho has also added a prevention part to safety planning. When a child can be safe at home, caseworkers use an in-home safety plan, called a prevention plan, to keep the child from entering foster care.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2017 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	No reporting requirement identified
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	Yes

Learn more: Summary of Idaho's Plan of Safe Care laws (Child Welfare Information Gateway)

Illinois

Profile last updated: October 2026

● **No Policy**[1]. No statewide Plan of Safe Care policy was identified as of 2024.

How it works in practice[1]
Illinois had no statewide Plan of Safe Care policy as of 2024. A state task force on family recovery plans first met in April 2025.

What the state calls the plan[1]	Not identified
Year the Plan of Safe Care policy began[1]	Not identified
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral for assessment or treatment
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	Yes. A positive newborn test is evidence of neglect in court hearings.
Child welfare accepts abuse or neglect reports during pregnancy[5]	Only when other children live in the home
Child welfare accepts referrals for services during pregnancy[5]	Yes

Learn more: Summary of Illinois's Plan of Safe Care laws (Child Welfare Information Gateway)

Indiana

Profile last updated: November 2026

- **All CPS Referrals**[1]. Every screened-in report for prenatal substance exposure gets a plan, developed during the child protective services assessment whether or not a case is opened.

How it works in practice[1][9]

A plan must be developed for every infant under age 1 identified as affected by legal or illegal substances, withdrawal, neonatal abstinence syndrome, or fetal alcohol spectrum disorder. It covers the health and treatment needs of the infant, parents, household members, and caregivers, and is developed whether or not the assessment is substantiated. If the case is opened and substantiated, or the family receives later reports, the plan must be updated until the child turns 1. The plan is also sent to the child's primary care provider.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2019 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral for assessment or treatment; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	Yes
Child welfare accepts abuse or neglect reports during pregnancy[5]	Yes
Child welfare accepts referrals for services during pregnancy[5]	Yes

Learn more: Summary of Indiana's Plan of Safe Care laws (Child Welfare Information Gateway)

Iowa

Profile last updated: October 2026

● **All CPS Referrals**[1]. Every screened-in report for prenatal substance exposure gets a plan, developed during the child protective services assessment whether or not a case is opened.

How it works in practice[1]
Iowa's federal reports do not mention Plans of Safe Care. Infants covered by the 2016 federal law are automatically referred to Early ACCESS, Iowa's early intervention program.

What the state calls the plan[1]	Not named in state reports
Year the Plan of Safe Care policy began[1]	2016 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Referral for assessment or treatment; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	Yes

Learn more: Summary of Iowa's Plan of Safe Care laws (Child Welfare Information Gateway)

Kansas

Profile last updated: October 2026

● **Alternative Response**[1]. Plans are delivered through a voluntary, in-home child welfare track, without a formal investigation or court involvement.

How it works in practice[1]
Families who need a Plan of Safe Care are referred to Family Preservation, which provides intensive in-home services to meet the needs listed in the plan.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2018 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral to child welfare

State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No. The definition was repealed in July 2025.
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	Yes

Learn more: Summary of Kansas's Plan of Safe Care laws (Child Welfare Information Gateway)

Kentucky

Profile last updated: October 2026

● **All CPS Referrals**[1]. Every screened-in report for prenatal substance exposure gets a plan, developed during the child protective services assessment whether or not a case is opened.

How it works in practice[1]
Health care providers report substance-affected newborns to child welfare and can include information about the mother's treatment with medication. Central intake screens each report. If a case is opened, the caseworker builds the plan into the case plan. If not, the plan becomes part of an aftercare plan.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2015 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Public health data
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of Kentucky's Plan of Safe Care laws (Child Welfare Information Gateway)

Louisiana

Profile last updated: October 2026

● **Dual Pathways**[1]. Some exposures, such as medication for opioid use disorder, lead to a

hospital-initiated plan. Other exposures require a report and a child protective services response.

How it works in practice[1]
When a newborn shows withdrawal or other effects from the mother's lawful use of a prescribed medication, the physician fills out a notification form that states no prenatal neglect is suspected, instead of calling the child abuse hotline. Other exposures go through the regular child protection response. The state tracks whether a plan was developed and referrals were made before discharge.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2017 (state law)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	Yes
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	Yes

Learn more: Louisiana DCFS: Substance-Exposed Newborns Reporting and Notifications · Summary of Louisiana's Plan of Safe Care laws (Child Welfare Information Gateway)

Maine

Profile last updated: October 2026

- **Alternative Response**[1]. Plans are delivered through a voluntary, in-home child welfare track, without a formal investigation or court involvement.

How it works in practice[1]
Maine has a Plan of Safe Care nurse who leads the work. Notifications of substance-exposed infants are tracked so that every infant receives a plan.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2016 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Referral for assessment or treatment; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No

Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	Yes

Learn more: Maine CDC: Plan of Safe Care · Summary of Maine's Plan of Safe Care laws (Child Welfare Information Gateway)

Maryland

Profile last updated: October 2026

● **Alternative Response**[1]. Plans are delivered through a voluntary, in-home child welfare track, without a formal investigation or court involvement.

How it works in practice[1]
Health care providers notify the local department of social services about substance-exposed newborns. A notification is not needed solely because a mother is legally using a prescribed medication, unless the baby shows effects. Health care providers, treatment providers, child protective services, and community organizations all help develop the plan.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2013 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	Yes, with conditions. Applies only to cocaine, heroin, or methamphetamine, and only if the parent does not take part in or complete treatment.
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Maryland Department of Human Services: Substance Exposed Newborn · Summary of Maryland's Plan of Safe Care laws (Child Welfare Information Gateway)

Massachusetts

Profile last updated: November 2026

● **Hospital-initiated**[1]. Health care or community providers develop the plan during pregnancy or

soon after birth, outside the child welfare system and before any decision about a report.

How it works in practice[8]

Substance use treatment, medical, and family service providers are expected to offer a plan to every patient with a substance-exposed pregnancy or birth. The family owns the plan, and it is tailored to their needs. The Department of Public Health leads this work.

What the state calls the plan[1]	Family Care Plan (also called Plan of Safe Care)
Year the Plan of Safe Care policy began[1]	2015 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Massachusetts Department of Public Health: Plan of Safe Care · Summary of Massachusetts's Plan of Safe Care laws (Child Welfare Information Gateway)

Michigan

Profile last updated: November 2026

● **All CPS Referrals**[1]. Every screened-in report for prenatal substance exposure gets a plan, developed during the child protective services assessment whether or not a case is opened.

Updated since the 2024 policy scan. Professionals in Michigan report that a plan is required for every substance-exposed infant referred to child protective services, so its approach is now shown as All CPS Referrals.[9]

How it works in practice[1][9]

A plan is required for every substance-exposed infant referred to child protective services, including infants exposed to THC (marijuana). Caseworkers develop the plan. Michigan's protocol covers three points in time, during pregnancy, at birth, and after birth, and is shared with child welfare staff, medical professionals, and service providers.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2017 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.

Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Michigan MDHHS: Plan of Safe Care · Summary of Michigan's Plan of Safe Care laws (Child Welfare Information Gateway)

Minnesota

Profile last updated: November 2026

● **Open Child Welfare Case**[1]. Plans are developed only after a family is found to need child welfare intervention. The plan works as a safety plan or case plan.

How it works in practice[1][9]

The Minnesota Hospital Association piloted a statewide plan template with hospitals and local child welfare agencies, for use during pregnancy or at birth. Under Minnesota Statutes section 260E.03, prenatal exposure to prescribed drugs, including methadone, is screened out and is not a mandated report.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2017 (state report)
Prescribed medication for opioid use disorder exempt from reporting[9]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Referral for assessment or treatment; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	Yes
Child welfare accepts abuse or neglect reports during pregnancy[5]	Yes
Child welfare accepts referrals for services during pregnancy[5]	Yes

Learn more: Summary of Minnesota's Plan of Safe Care laws (Child Welfare Information Gateway)

Mississippi

Profile last updated: October 2026

● **Alternative Response**[1]. Plans are delivered through a voluntary, in-home child welfare track,

without a formal investigation or court involvement.

How it works in practice[1]
We could not find a detailed public description of how Mississippi's process works. Under its approach, plans are developed through a child welfare track that offers voluntary, in-home services without a formal investigation.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2015 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	No reporting requirement identified
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of Mississippi's Plan of Safe Care laws (Child Welfare Information Gateway)

Missouri

Profile last updated: October 2026

- **Alternative Response**[1]. Plans are delivered through a voluntary, in-home child welfare track, without a formal investigation or court involvement.

How it works in practice[1]
Children's Division staff first assess whether a plan is needed, for example when an infant tests positive or shows withdrawal, or when a parent's continued substance use may affect the baby's safety. A plan is developed only when this assessment finds a risk.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2017 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No

Child welfare accepts referrals for services during pregnancy[5]	Yes

Learn more: Summary of Missouri's Plan of Safe Care laws (Child Welfare Information Gateway)

Montana

Profile last updated: October 2026

● **Alternative Response**[1]. Plans are delivered through a voluntary, in-home child welfare track, without a formal investigation or court involvement.

How it works in practice[1]

Montana treats legally and illegally obtained drugs the same way, but a report is not required when an infant is affected only by methadone or buprenorphine prescribed by a provider. Plans are developed through a voluntary child welfare track.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2020 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of Montana's Plan of Safe Care laws (Child Welfare Information Gateway)

Nebraska

Profile last updated: October 2026

● **Hospital-initiated**[1]. Health care or community providers develop the plan during pregnancy or soon after birth, outside the child welfare system and before any decision about a report.

How it works in practice[1]

Nebraska uses a Plan of Safe Care at birth and also a Prenatal Plan of Safe Care that can be made during pregnancy.

What the state calls the plan[1]	Plan of Safe Care; Prenatal Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2019 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified

Purpose of required reports of drug use during pregnancy[3]	No reporting requirement identified
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	Yes
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of Nebraska's Plan of Safe Care laws (Child Welfare Information Gateway)

Nevada

Profile last updated: October 2026

● **Hospital-initiated**[1]. Health care or community providers develop the plan during pregnancy or soon after birth, outside the child welfare system and before any decision about a report.

How it works in practice[1]

Nevada calls it a CARA Plan of Care. Providers use a shared definition of a substance-affected infant to decide when a plan is required. The definition includes infants whose mothers are in treatment for a substance use disorder.

What the state calls the plan[1]	CARA Plan of Care
Year the Plan of Safe Care policy began[1]	2021 (state law)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Referral for assessment or treatment; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of Nevada's Plan of Safe Care laws (Child Welfare Information Gateway)

New Hampshire

Profile last updated: October 2026

● **Hospital-initiated**[1]. Health care or community providers develop the plan during pregnancy or

soon after birth, outside the child welfare system and before any decision about a report.

How it works in practice[1]
State law names health care providers as the main developers of the plan. Providers notify the Division for Children, Youth and Families when an infant is affected by substance use, withdrawal, or fetal alcohol spectrum disorder.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2018 (state law)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Referral for assessment or treatment
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of New Hampshire's Plan of Safe Care laws (Child Welfare Information Gateway)

New Jersey

Profile last updated: November 2026

- **All CPS Referrals**[1]. Every screened-in report for prenatal substance exposure gets a plan, developed during the child protective services assessment whether or not a case is opened.

How it works in practice[1][9]
A team of child welfare staff, a drug and alcohol counselor, a domestic violence liaison, a behavioral health consultant, and an early childhood liaison advises the caseworker. The caseworker then develops the plan with the family and follows up on its recommendations. New Jersey recently renamed its Plan of Safe Care the Family Care Plan. Child welfare can also accept a referral during pregnancy for a service assessment to connect the mother to services.

What the state calls the plan[1]	Family Care Plan (formerly Plan of Safe Care)
Year the Plan of Safe Care policy began[1]	2019 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Referral for assessment or treatment; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	Yes

Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[9]	Yes. Child welfare can do a service assessment during pregnancy to connect the mother to services

Learn more: New Jersey DCF: Plans of Safe Care brochure (PDF) · Summary of New Jersey's Plan of Safe Care laws (Child Welfare Information Gateway)

New Mexico

Profile last updated: October 2026

● **Hospital-initiated**[1]. Health care or community providers develop the plan during pregnancy or soon after birth, outside the child welfare system and before any decision about a report.

New Mexico substantially changed its mandated reporting requirements in 2025. The reporting details below may not reflect those changes. Its approach is still hospital-initiated.

How it works in practice[6]

Hospitals send either a notification or a report, as in Connecticut, but notifications include the family's identifying information. Plan information is kept separate from the main child welfare data system. Medicaid managed care organizations coordinate care after the hospital stay and receive plans automatically through the state portal, and a navigator helps connect child welfare staff with care coordinators. State law (House Bill 230, 2019) gives managed care organizations this role. In 2025, the state issued a directive requiring the child welfare agency to seek custody of certain substance-exposed newborns.

What the state calls the plan[1]	Plan of Care
Year the Plan of Safe Care policy began[1]	2019 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral for assessment or treatment
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	Unclear

Learn more: New Mexico Health Care Authority: CARA Plan of Safe Care · Summary of New Mexico's Plan of Safe Care laws (Child Welfare Information Gateway)

New York

Profile last updated: October 2026

● **Open Child Welfare Case**[1]. Plans are developed only after a family is found to need child welfare intervention. The plan works as a safety plan or case plan.

How it works in practice[1]

New York is putting Plans of Safe Care in place in three stages. Health care providers notify the child protective services system about affected infants.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2011 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral for assessment or treatment
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of New York's Plan of Safe Care laws (Child Welfare Information Gateway)

North Carolina

Profile last updated: October 2026

● **Alternative Response**[1]. Plans are delivered through a voluntary, in-home child welfare track, without a formal investigation or court involvement.

How it works in practice[8]

Child welfare refers the infant to Care Management for At-Risk Children, a local care management program. The care manager does a health assessment, links the baby to a medical home, and connects the family to services such as parenting programs, home visiting, mental health care, housing, and early intervention.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2017 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	No reporting requirement identified
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No

Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: North Carolina DHHS: Infant Plan of Safe Care · Summary of North Carolina's Plan of Safe Care laws (Child Welfare Information Gateway)

North Dakota

Profile last updated: October 2026

● **Alternative Response**[1]. Plans are delivered through a voluntary, in-home child welfare track, without a formal investigation or court involvement.

How it works in practice[1]
The plan applies to infants younger than 28 days at the time of a report who are affected by substance use, withdrawal, or fetal alcohol spectrum disorder. Plans are developed through a voluntary child welfare track.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2017 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral for assessment or treatment
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No. The definition covers prenatal substance use or exposure, not a test result.
Child welfare accepts abuse or neglect reports during pregnancy[5]	Yes
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of North Dakota's Plan of Safe Care laws (Child Welfare Information Gateway)

Ohio

Profile last updated: October 2026

● **All CPS Referrals**[1]. Every screened-in report for prenatal substance exposure gets a plan, developed during the child protective services assessment whether or not a case is opened.

How it works in practice[1]
Plans are for caregivers and families of infants exposed to legal or illegal substances before birth. A report is not required for methadone, buprenorphine, or other medications taken as prescribed.

What the state calls the plan[1]	Plan of Safe Care

Year the Plan of Safe Care policy began[1]	2018 (state law)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Public health data
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of Ohio's Plan of Safe Care laws (Child Welfare Information Gateway)

Oklahoma

Profile last updated: October 2026

● **All CPS Referrals**[1]. Every screened-in report for prenatal substance exposure gets a plan, developed during the child protective services assessment whether or not a case is opened.

How it works in practice[6]
A statewide initiative led by the state behavioral health agency, working with child welfare and health care providers, helps families make a "family care plan" during pregnancy. It supports families from pregnancy through early childhood. The initiative started as a pilot and is not yet statewide, so most families still go through the state's child welfare process.

What the state calls the plan[1]	Not identified
Year the Plan of Safe Care policy began[1]	2013 (state law)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral for assessment or treatment; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	Yes, with conditions. Applies when an assessment also finds the infant at risk of harm.
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of Oklahoma's Plan of Safe Care laws (Child Welfare Information Gateway)

Oregon

Profile last updated: November 2026

● **Hospital-initiated**[1]. Health care or community providers develop the plan during pregnancy or soon after birth, outside the child welfare system and before any decision about a report.

Updated since the 2024 policy scan. Oregon does not require health care providers to report prenatal drug use, and families can make a Family Care Plan through a voluntary community pathway, so its approach is now shown as hospital-initiated. Child welfare still makes sure a plan exists when it is involved with a family, unless the family already has one or was already offered one.[9]

How it works in practice[8][9][6]

Health care providers are not required to report prenatal drug use to child protective services. A report is made only when there is reasonable suspicion of abuse or neglect. Instead, providers connect families to voluntary services, ideally a peer support specialist or other support worker, as early as possible and at least at birth. Family Care Plans are developed by community service professionals outside the medical system, through a statewide voluntary pathway led by Comagine Health, the Oregon Health Authority, and the Oregon Department of Human Services, and families can also create their own plans. Under state law (ORS 430.920), providers share information with the local public health administrator without identifying the family, including whether a plan was developed and whether referrals were made. When child welfare is involved, it makes sure a plan is created during its assessment, unless the family already has one or was already offered one. Nurture Oregon also serves pregnant people with substance use disorder who have Medicaid in six counties.

What the state calls the plan[1]	Plan of Care; Family Care Plan (voluntary, during pregnancy)
Year the Plan of Safe Care policy began[1]	2018 (state law)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral for assessment or treatment
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Oregon Family Care Plans (Comagine Health, Oregon Health Authority, and Oregon Department of Human Services) · Summary of Oregon's Plan of Safe Care laws (Child Welfare Information Gateway)

Pennsylvania

Profile last updated: November 2026

● **All CPS Referrals**[1]. Every screened-in report for prenatal substance exposure gets a plan, developed during the child protective services assessment whether or not a case is opened.

Updated since the 2024 policy scan. Professionals in Pennsylvania report that a ChildLine report and an investigation are always required, so its approach is now shown as All CPS Referrals instead of Alternative Response.[9]

How it works in practice[1][9]

A report to ChildLine and an investigation are required. Before the baby leaves the hospital, a team meets to shape the plan. The team can include public health, home visiting, substance use and mental health treatment, child welfare, early intervention, schools, insurers, and the hospital. After the investigation, some families are diverted to other programs, but many cases still go through the traditional child welfare and court process.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2019 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Referral for assessment or treatment
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of Pennsylvania's Plan of Safe Care laws (Child Welfare Information Gateway)

Rhode Island

Profile last updated: October 2026

● **Hospital-initiated**[1]. Health care or community providers develop the plan during pregnancy or soon after birth, outside the child welfare system and before any decision about a report.

How it works in practice[8]

Rhode Island calls its plan the Circle of Care Plan. Birthing hospitals must offer one to the caregivers of substance-exposed newborns. The Department of Health's Perinatal Substance Use program manages the plans, which record referrals for the needs of both the baby and the caregivers.

What the state calls the plan[1]	Circle of Care Plan (also called Plan of Safe Care)
Year the Plan of Safe Care policy began[1]	2018 (state report)

Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	Only when other children live in the home
Child welfare accepts referrals for services during pregnancy[5]	Yes

Learn more: Rhode Island DCYF: Circle of Safe Care Plan · Summary of Rhode Island's Plan of Safe Care laws (Child Welfare Information Gateway)

South Carolina

Profile last updated: October 2026

● **Open Child Welfare Case**[1]. Plans are developed only after a family is found to need child welfare intervention. The plan works as a safety plan or case plan.

How it works in practice[1]
We could not find a detailed public description of how South Carolina's process works. South Carolina calls these Family Wellness and Support Plans, and plans are developed for families who have an open child welfare case.

What the state calls the plan[1]	Family Wellness and Support Plan
Year the Plan of Safe Care policy began[1]	2018 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Public health data
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	Yes, with conditions. Applies when another of the mother's children also tested positive or was diagnosed with fetal alcohol syndrome.
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of South Carolina's Plan of Safe Care laws (Child Welfare Information Gateway)

South Dakota

Profile last updated: October 2026

● **Open Child Welfare Case**[1]. Plans are developed only after a family is found to need child welfare intervention. The plan works as a safety plan or case plan.

How it works in practice[1]
Child protective services works with the Department of Health to encourage referrals of substance-exposed infants to the state's Plans of Safe Care program through the Helpline Center.

What the state calls the plan[1]	Plan of Safe Care (also called Safe Care Plan)
Year the Plan of Safe Care policy began[1]	2018 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No. The definition covers prenatal substance use or exposure, not a test result.
Child welfare accepts abuse or neglect reports during pregnancy[5]	Yes
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of South Dakota's Plan of Safe Care laws (Child Welfare Information Gateway)

Tennessee

Profile last updated: October 2026

● **Open Child Welfare Case**[1]. Plans are developed only after a family is found to need child welfare intervention. The plan works as a safety plan or case plan.

How it works in practice[1]
The Department of Children's Services works with the Governor's Children's Cabinet, a group of state agencies that is using a "single team, single plan" approach to develop plans for substance-exposed infants.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2018 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	No reporting requirement identified

State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of Tennessee's Plan of Safe Care laws (Child Welfare Information Gateway)

Texas

Profile last updated: October 2026

- **Open Child Welfare Case**[1]. Plans are developed only after a family is found to need child welfare intervention. The plan works as a safety plan or case plan.

How it works in practice[1]

If a plan was made before child welfare became involved, the caseworker uses it in the family's assessment and talks with the providers who helped during pregnancy. When the Department of Family and Protective Services develops the plan itself, it does so case by case.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2015 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Public health data
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No. The definition covers prenatal substance use or exposure, not a test result.
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Summary of Texas's Plan of Safe Care laws (Child Welfare Information Gateway)

Utah

Profile last updated: October 2026

- **Open Child Welfare Case**[1]. Plans are developed only after a family is found to need child

welfare intervention. The plan works as a safety plan or case plan.

How it works in practice[5]

Utah calls its plan a Child and Family Plan, a safety plan created by the Division of Child and Family Services. When child welfare receives a referral about a pregnant woman, it sends the referral to the state's substance use treatment agency. That agency reaches out to the woman directly to offer voluntary services. Utah's Office of Substance Use and Mental Health also works with other state partners to expand the use of Plans of Safe Care.

What the state calls the plan[1]	Child and Family Plan (also called Plan of Safe Care)
Year the Plan of Safe Care policy began[1]	2017 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	Referred to the state substance use treatment agency, which offers voluntary services

Learn more: Summary of Utah's Plan of Safe Care laws (Child Welfare Information Gateway)

Vermont

Profile last updated: October 2026

● **Dual Pathways**[1]. Some exposures, such as medication for opioid use disorder, lead to a hospital-initiated plan. Other exposures require a report and a child protective services response.

How it works in practice[1]

Vermont uses two paths. For infants exposed to certain substances, such as medication for opioid use disorder, hospitals send a notification without names and health care providers make the plan with no child welfare contact. Other exposures require a report and a child welfare response.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2020 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	No reporting requirement identified

State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	Yes
Child welfare accepts referrals for services during pregnancy[5]	No

Learn more: Vermont DCF: Newborn Plan of Safe Care FAQ (PDF) · Summary of Vermont's Plan of Safe Care laws (Child Welfare Information Gateway)

Virginia

Profile last updated: November 2026

● **Open Child Welfare Case**[1]. Plans are developed only after a family is found to need child welfare intervention. The plan works as a safety plan or case plan.

Updated since the 2024 policy scan. Professionals in Virginia report that a plan is completed only for reports that are screened in and opened as a case, so its approach is now shown as Open Child Welfare Case.[9]

How it works in practice[9]
A Plan of Safe Care is completed only for reports that child protective services screens in and opens as a case. Prescribed medication for opioid use disorder is not exempt from reporting in Virginia.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2021 (state law)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Referral for assessment or treatment; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	In some local areas

Learn more: Summary of Virginia's Plan of Safe Care laws (Child Welfare Information Gateway)

Washington

Profile last updated: October 2026

● **Hospital-initiated**[1]. Health care or community providers develop the plan during pregnancy or

soon after birth, outside the child welfare system and before any decision about a report.

Updated since the 2024 policy scan. Washington's community pathway began as a pilot in two counties in 2021 and was rolled out to birthing hospitals across the state through 2024, so its approach is now shown as hospital-initiated.[7]

How it works in practice[6]

Hospitals use an online portal to decide whether there is a safety concern. If there is none, they send an anonymous notification, and the family is offered voluntary support from a statewide community-based organization. That organization is funded by child welfare but works outside it, connecting families with services, benefits, infant supplies, and family resource centers. If there is a safety concern, hospitals make a formal report.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2014 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	No reporting requirement identified
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	No, but offers resource information

Learn more: Washington DCYF: Plan of Safe Care FAQs · Summary of Washington's Plan of Safe Care laws (Child Welfare Information Gateway)

West Virginia

Profile last updated: October 2026

- **All CPS Referrals**[1]. Every screened-in report for prenatal substance exposure gets a plan, developed during the child protective services assessment whether or not a case is opened.

How it works in practice[1]

We could not find a detailed public description of how West Virginia's process works. Under its approach, every infant reported to child protective services for prenatal substance exposure gets a plan, developed during the assessment.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2015 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.

Purpose of required reports of drug use during pregnancy[3]	No reporting requirement identified
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	Yes

Learn more: Summary of West Virginia's Plan of Safe Care laws (Child Welfare Information Gateway)

Wisconsin

Profile last updated: October 2026

● **Alternative Response**[1]. Plans are delivered through a voluntary, in-home child welfare track, without a formal investigation or court involvement.

How it works in practice[1]

Wisconsin's child protective services standards spell out how agencies develop, monitor, and follow up on the plan, which is treated as part of the family's safety plan.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2004 (state report)
Prescribed medication for opioid use disorder exempt from reporting[2]	Yes. A report is not required when the only exposure is to prescribed medication, such as methadone or buprenorphine, taken as prescribed.
Purpose of required reports of drug use during pregnancy[3]	Public health data; Referral to child welfare
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No. The definition covers prenatal substance use or exposure, not a test result.
Child welfare accepts abuse or neglect reports during pregnancy[5]	Yes
Child welfare accepts referrals for services during pregnancy[5]	Unclear

Learn more: Summary of Wisconsin's Plan of Safe Care laws (Child Welfare Information Gateway)

Wyoming

Profile last updated: October 2026

● **Hospital-initiated**[1]. Health care or community providers develop the plan during pregnancy or

soon after birth, outside the child welfare system and before any decision about a report.

How it works in practice[8]
Wyoming Patient Care Teams, which can include medical staff, treatment providers, mental health professionals, peer specialists, and early childhood staff, develop the plan with the family. It is best started during pregnancy and must be finished before the mother leaves the hospital.

What the state calls the plan[1]	Plan of Safe Care
Year the Plan of Safe Care policy began[1]	2024 (state law)
Prescribed medication for opioid use disorder exempt from reporting[2]	No exemption identified
Purpose of required reports of drug use during pregnancy[3]	Public health data
State's definition of abuse or neglect includes a positive infant drug or alcohol test[4]	No
Child welfare accepts abuse or neglect reports during pregnancy[5]	No
Child welfare accepts referrals for services during pregnancy[5]	Unclear

Learn more: Wyoming Department of Family Services: Plans of Safe Care · Summary of Wyoming's Plan of Safe Care laws (Child Welfare Information Gateway)

Sources

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3. Purpose of required reports: National Institute on Alcohol Abuse and Alcoholism, Alcohol Policy Information System. Reporting Requirements.
4. Definition of abuse or neglect: National Institute on Alcohol Abuse and Alcoholism, Alcohol Policy Information System. Legal Significance for Child Abuse/Child Neglect. Policies as of January 1, 2025, with our review of each state's law.
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6. How it works in practice (Connecticut, New Mexico, Oklahoma, Oregon, and Washington): Ganetsky, V. S., Feder, K. A., Xu, J., Kennedy-Hendricks, A., & Bandara, S. (2026). Innovations in plans of safe care

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7. Washington update: Washington State Department of Children, Youth, and Families. Plan of Safe Care implementation schedule.
8. How it works in practice (Colorado, Massachusetts, North Carolina, Oregon, Rhode Island, and Wyoming): each state's official Plan of Safe Care page, linked under "Learn more" in that state's profile.
9. Updates from people in the state: corrections and confirmations from child welfare and health care professionals who reviewed this site in November 2026.

This summary is for education and research. It is not legal advice. Suggested citation: Lloyd Sieger, M. (2026). *Plan of Safe Care: State policies, research, and help for families*. https://planofsafecare.org/